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Dutch Healthcare Authority  
 Attn. Chairwoman, Ms. G. Engwirda  
 P.O. Box 3017  
 3502 GA Utrecht

Date

February 13, 2024

Our reference

z2023-3000866

Your letter dated

November 1, 2023

Contact person

Subject

Advice on the Regulation on Nursing and Care Advice on the Regulation on Nursing and Care  
 NR/REG-2407

Dear Ms. Engwirda,

On November 1, 2023, the Dutch Healthcare Authority (hereinafter: NZa) requested the Authority for Personal Data (hereinafter: AP) to provide advice on the Regulation on Nursing and Care, NR/REG-2407 (hereinafter: Regulation). This request for advice particularly concerns the new provisions included in the Regulation regarding the processing of personal data related to the so-called client profiles in home care. For the sake of clarity, the AP notes that in this case – by way of exception – advice has been provided on an established and published text of a regulation. However, advice from the AP should be requested during the drafting phase of regulations, even if there is still room for consideration of the AP's advice between the moment of publication and the actual entry into force of the regulation.

Assessment of the personal data aspects of the Regulation gives the AP reason to make some comments.

#### Scope of the Regulation

The provisions regarding client profiles in home care included in this Regulation are part of an experiment initiated in 2021 (pursuant to Article 58 Wmg) in which the NZa, in collaboration with home care nurses, care providers, and health insurers, aims for a new method of financing home care. Since 2022, experience has been gained in the context of this experiment with working with client profiles<sup>1</sup>. These experiences<sup>2</sup> and the conclusions drawn from them form the basis for

<sup>1</sup> This experiment runs until December 31, 2028.

<sup>2</sup> These efforts, experiences, and considerations are described in the Accountability Document for the client profiles experiment in nursing & care from 2024 (June 2023).

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provisions regarding the inclusion of client characteristics related to a Capacity/Load model (hereinafter DKDL-model)<sup>3</sup> in the administration of the care provider. For clients<sup>4</sup>, a number of questions regarding capacity, load, and support needs must be answered. The answers are important

for processing the claim but also provide insight into the care needs of clients, which can also be useful from market regulation considerations. In the Electronic Client Dossier, the client profile is derived from the answers<sup>5</sup>. The Regulation contains the obligation for the care provider to provide the client profile to the health insurer when submitting the claim. Subsequently, these client profiles will also be made available to the NZa via Vektis and Zorg TTP.

#### Advice

1. Justification of necessity, proportionality, and subsidiarity of the mandatory nature and scope of the processing of (special) personal data.

The AP notes that the NZa does not provide substantive justification in the explanation for the necessity, proportionality, and subsidiarity of the imposed obligations for the processing of (special) personal data regarding the client profiles. In that explanation, reference is made (only) to the NZa Accountability Document Reporting on the accountability of financing client profiles in nursing and care. The AP observes that in documents such as the Accountability Document for the client profiles experiment in nursing & care from 2024, the (extracted) summary from the DPIA provided to the AP in the context of the advice request, and the (advice from the FG provided to the AP in the context of the advice request) regarding the conducted DPIA, the necessary considerations and assessments are included that can substantiate the necessity, proportionality, and subsidiarity.

The AP advises to supplement the explanation with the relevant considerations and assessments derived from the underlying documents.

2. Inclusion of a possibility to prevent the provision of the client profile to the health insurer and the NZa through a privacy statement.

The Regulation (Article 6.7) provides for the exemption from the information obligation of the care provider when submitting the claim if a privacy statement is present. The explanation accompanying the Regulation does not further indicate why this possibility is also made available for the client group in this sector.<sup>6</sup>

The AP advises to clarify in a substantive explanation the considerations that necessitate the introduction of this possibility for this sector and category of stakeholders.

3 For an explanation and justification of the DKDL-model, reference is made to the NZa accountability document 'Reporting – accountability of financing client profiles in nursing and care. Available at: [https://puc.overheid.nl/nza/doc/PUC\\_745371\\_22/1](https://puc.overheid.nl/nza/doc/PUC_745371_22/1).

4 For all clients, so-called 'initial questions' must be registered, which are intended to exclude certain client groups (namely children up to 18 years, palliative-terminal clients) from the DKDL-model.

5 In this model, there are 12 client profiles based on 9 questions in a so-called case-mix questionnaire.

6 Based on the relevant ruling of the CvBB from 2013 regarding the privacy statement, such an option must – at least – exist for clients in the mental health sector, but is not necessarily required to be offered to clients in other sectors.

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#### 3. Evaluation of the experiment using client profiles

The main objective of the experiment is to base the funding of district nursing (more) on the characteristics of the care demand/needs of clients. Starting in 2024, care providers and health insurers will have the opportunity to use information based on client profiles in the contracting of care. It is expected that this will lead to better alignment between care demand and supply. The NZa intends to measure this use after some time (likely in 2027). For this purpose, the NZa may also, in addition to other methods (such as interviews, etc.), wish to utilize the data regarding client profiles that the NZa already has access to in the context of its statutory duty to supervise the legality and efficiency of

billing. It is recognized that the evaluation of the experiment will involve (secondary) use of that data for a different task, namely monitoring the functioning of the care market and the operation of the care purchasing market, than for which that data was primarily obtained. In principle, the NZa, based on the relevant provisions in the Wmg, is authorized to use data obtained for one statutory task also for the fulfillment of other tasks.

The AP advises that in the case of - possibly - secondary use of these client profiles for district nursing for that evaluation, a clearly formulated and delineated question should be employed, and against that background, the necessity, proportionality, and subsidiarity of that data processing should be carefully weighed and substantiated in advance.

#### 4. Information for stakeholders regarding data processing in the context of the use of client profiles for the funding of district nursing

Also in the context of this experiment with client profiles for district nursing, it is important that the parties involved in the data processing (care providers, health insurers, and NZa) ensure the necessary coordination regarding and design of their respective responsibilities for providing information about this to the stakeholders. The care provider, as the entity collecting data from the stakeholder, has an extensive obligation to provide information under Article 13 of the GDPR. However, the recipients (health insurers and NZa) must also reflect on what can be expected of them under Article 14 of the GDPR. If the involved parties are not sufficiently informed about what other parties are doing in this regard, gaps and/or contradictions in the information provided to stakeholders may arise. A suitable, unambiguous, and neutral description by the parties of the purposes of the processing and the rights that stakeholders can exercise against the parties is required.

The AP advises that the explanation should outline how the NZa will inform care consumers about the tasks of the NZa, the associated data processing, and the rights they have against the NZa.

#### 7 Data Protection Impact Assessment Experiment Client Profiles District Nursing from 2024.

8 See also the advice of the FG regarding the DPIA in this regard.

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Workload AP

The AP notes that the draft is not expected to require additional effort from the AP.

Disclosure

This advice will be published on the AP's website within two weeks.

Sincerely,

Authority Personal Data,

Aleid Wolfsen

Chairman